

OUTLINE OF EVIDENCE – MR CORY LEA SHEPHERD (APPELLANT)

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

I will give evidence of the following. Paragraph references are to the notice to admit facts served on the Respondent on 28 August 2026 (the Notice), whose Annexure A holds the documents referred to.

1. **Employment and the role.** I commenced at Logan Hospital Switchboard on 25 March 2019 as a casual, became permanent from 3 March 2021 and full time from 16 October 2023, as a continuous shift worker. The duties: emergency response notifications distributed under the code procedures "strictly adhering to protocols and timeframes"; the pager and on-call records on which emergency paging depends; and some 200 to 300 calls a shift, including urgent clinical handover and distressed patients and families. [Notice ¶¶ 1–16, 26–38]
2. **Sleep and fatigue.** My sleep on rotating shifts across 2023 and 2024 and the melatonin and temazepam prescribed on 16 November 2023. The shifts of 17 and 18 March 2024: my travel time each way, finishing at 23:00 and starting at 06:00, the rest and sleep actually available, the emergency codes I handled on 18 March, and the sick leave of 19 March. The fatigue concerns I raised; the toolkit sent to Ms Taylor with no response; my request of 8 April 2024 and the refusal of 1 May 2024; and that before 30 June 2024 I had no fatigue training, no fatigue or psychosocial risk assessment of my rostering, and no fatigue management process. [¶¶ 17–25, 211–256, 263–268]
3. **Rostering: lates then earlies.** The pattern on my roster line of late shifts followed by early shifts, and of nights followed by days off and then nights again, which I raised on 16 April 2024; that rostering errors on my line recurred across successive roster periods; and Ms Reese's acknowledgement of 10 May 2024 of "a few rostering errors made by Chloe with regards to Cory's line in past rosters". [¶¶ 211–223]
4. **Pay and entitlements.** The public holidays I was not rostered between February and April 2024 and the difference to my pay; that Payroll identified the errors on 3 May 2024 and directed my manager to submit an AVAC for each fortnight; that I could not submit one myself; what I was told between 3 and 28 May 2024; and the validation of claims older than three months I was asked to sign on 28 May. The special pandemic leave application of February 2024: what I attached, the two declines, and the approval on the same material. [¶¶ 114–142, 182–210]
5. **The database, the misdirected calls, and what I could not fix.** That from 18 July 2023 operators could no longer correct database entries and the Contact & Number Changes book was removed, so a wrong number waited on Ms Stibbard or Ms Taylor and after hours had to "wait until either Chloe or myself are back"; the directives of 15 April, 19 April and 9 May 2024, and that I was not consulted before any of them. That from 15 April 2024, when Ms Taylor notified the department that she and Ms Stibbard had placed themselves on after-hours call, departments and clinicians ringing during my shifts with pager and phone updates asked for her, and on many occasions neither they nor I could have the update or issue resolved during the shift. What I saw at the console between 2 and 8 May 2024: the nine occasions the MASPER Registrar reported of calls reaching the wrong team, including the MET call team ringing to ask where the MET call was because "switchboard could not tell them where VHUB was"; the Integrated Respiratory Service writing on 15 May and again on 20 May 2024 that calls were still being put through to them and "we can not help patients or other clinical staff"; that on each occasion I could not correct the entry myself and the delay built up while the correction waited; my recommendation of 20 May 2024 that the clinic contact document be reviewed; and Ms Taylor's statement to the department on 9 May 2024 of "many ongoing issues raised by the MASPER and the medical department about calls being transferred to the wrong medical teams". [¶¶ 39–113]
6. **Ms Taylor's hours: the question, the retraction request, and the answer.** That neither I nor the department knew her office hours, though directed to contact her through them; my email of 15 May 2024 at 1:15 pm asking her to share them and that changes be made in consultation with the team; Ms Reese's reply that evening that the email did not comply with the Code of Conduct and asking me to retract it, which I did; that two days later, on 17 May 2024 at 9:30 am, Ms Taylor sent the whole department an email headed "Switchboard Manager - On call and Hours." stating "Otherwise my hours are from 06:30-14:30", the very information I had asked for; and Ms Reese's reply of 21 May 2024 asking me to identify the directives I was concerned about. [¶¶ 70–88]
7. **The book, the earlier concerns, and the delegate role.** The communication book entry of 6 June 2023 and its removal; the concerns I raised on 7 August 2023 and the responses; my complaint of 13 May 2024, later determined to be a public interest disclosure (content not set out); and my interest in becoming the Switchboard union delegate and what followed. [¶¶ 143–181, 283–299]
8. **My health before June 2024.** That before 18 June 2024 I had never been diagnosed with or treated for depression. My general practitioner's record of 16 November 2023 notes poor sleep with shift work, that I could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good. The ADHD and anxiety noted in my history in October 2022 and the specialist referral renewed on 16 May 2024, and what each concerned. [General-practice records disclosed by the Respondent]
9. **Onset, presentation and diagnosis.** The symptoms I experienced from about 18 June 2024 and their effect on my sleep, concentration, self-care and relationships. Attending Dr Slawinski on 28 June 2024, whose note records "stress at work" and "upset by people not following rules", and Dr Hawes on 1 July 2024, whose note records "work stress", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies", and "they don't listen to his complaints". Lodging my application for compensation on 1 July 2024. Dr Hawes's work capacity certificates of 1 July and 8 September 2024 and his referral to a psychiatrist. My first consultation with Dr Krishnaiah on 24 October 2024, on that referral, and my treatment since. The matters at paragraphs 2 to 7 above are the matters I reported to each doctor, as their records show. I offer no clinical or diagnostic opinion.
10. **After 18 June 2024, and documents.** Meetings scheduled while I was on certified leave; that my employment was ended under abandonment provisions in October 2024 while I held current medical certificates, and my later reinstatement; my return on reduced hours and the leave taken since; that my Queensland Health email account was restricted, so I cannot produce my work emails myself. I will produce the rosters and payslips, my emails and messages with my line manager and the Director, the leave and payroll records produced by the employer, the role description, the medical records referred to above, and the letter of Metro South Hospital and Health Service of 5 June 2026 (ref K-LM26/729).

This outline states the topics on which I will give oral evidence. It is not a statement of evidence and is not verified.